

37. THE PRACTICAL BRAIN

DURATION : 13:08

STUDY SHEET

COURSE SUMMARY

This video, 'The Practical Brain', presents a detailed biodynamic approach centred on the brain and its expansion. The instructor teaches how to identify levels of expansion in the patient, exploring effective revitalisation strategies, such as working on the heart and digestive axes. This is followed by an exploration of cephalic and pontine flexion movements, with specific precautions to be taken during touch. By focusing on the sensation of expansion, practitioners will learn to accompany the movement of encephalisation, understanding the rhythm and synchronicity linked to biodynamic embryology. This session is both theoretical and practical, promising students an enriched understanding and developed skills to improve their practice.

37. THE PRACTICAL BRAIN: A BIODYNAMIC APPROACH

I. INITIAL POSITIONING AND SEEKING EXPANSION

I position myself above the patient, specifically at the level of the **insula**. My hands are placed here, my thumbs slightly crossing, and I ensure I have good support for my elbows and wrists on the table.

The **touch** is extremely gentle. I aim to feel the **expansion** of the field.

A. IDENTIFYING LACK OF EXPANSION

The very first thing to assess is the level of **expansion**. If the expansion is insufficient and I am not sufficiently "pushed back", it is a sign that something is amiss.

Often, one expects an immediate and strong expansion, but this is not always the case.

B. REVITALISATION STRATEGIES IN CASE OF INSUFFICIENT EXPANSION

If expansion is insufficient, several avenues can be explored to **revitalise** the system:

- **The heart:** Easing the heart is often a very effective approach.
- **Digestive axes:** Working on the digestive axes (mesenteric, mesaric) or the coeliac axis can bring the necessary expansion.

- **The sacrum and pedicles:** Moving up to the sacrum and pedicular levels can also help.

Generally, working with the heart or digestive axes yields quicker and more effective results.

II. THE FLEXION MOVEMENT AND ENCEPHALISATION

Once expansion is present, I feel an expansion followed by a **flexion** movement forwards. This is cephalic flexion, which brings the brain towards the heart.

Behind, I must perceive a **cervical flexion** movement. If I feel a slight deficit at this level, it indicates the need for a cervical or structural correction.

A. PONTINE FLEXION AND TELENCEPHALISATION

I then focus on my little fingers and seek **pontine flexion**. The movement descends.

Then, a new expansion appears. At this stage, I can open my hand. This is the beginning of the **telencephalisation** process, which is recovered from the occipital to the temporal level.

B. COMMON ERRORS AND PRECAUTIONS

Most errors occur when directly touching the brain. It is imperative not to touch the brain if one is not "under that frequency".

- I position myself above, without touching with my thumbs, maintaining a good distance.
- I create a very gentle **fulcrum** with my thumbs and a significant point of support.

C. THE SENSATION OF EXPANSION

Good expansion is felt as a plane of warmth and a gentle pushing sensation.

III. SEQUENTIAL MOVEMENT AND ACCOMPANIMENT

I perform a very gentle movement. This is not yet my "insular H-bridge". I make the flexion movement forwards. The movement descends well, but can slightly twist (for example, to the left). I very gently accompany this twist.

At this stage, a small correction may be necessary, potentially at the cardiac level. There may be a moment when the movement is not perfect, but that is not a problem.

A. DESCENT OF FLEXION AND PONTINE PLANE

I then move towards the descent of flexion, which the patient should feel distinctly. It descends without issue.

Once this is established, I move to the **pontine plane**. Here too, a small intervention may be required.

B. THE TELENCEPHALIC PLANE AND ASCENDING EXPANSION

After the pontine plane, I shift my attention to the **telencephalic plane**, the telencephalic anlage. The movement should come towards me. I observe an expansion.

I position myself on the "**Hitchman**": the movement turns towards me, expands, and ascends.

C. THE GREAT MOVEMENT OF ENCEPHALISATION

The movement restarts, descends again, and expresses itself. I very gently descend into the great flexion, the great movement of **encephalisation**. I recover this movement at the occipital level.

It is crucial to accompany this movement, not to impose it. We wait for it to manifest, then we follow it. The movement is already present, like a "drawn track", with a particular rhythm. We only reproduce the existing **pattern**.

IV. UNDERSTANDING RHYTHM AND SYNCHRONICITY

This pattern is not reproducible in the sense of a periodically repeating cycle. It is a system that is constantly there, a **writing in the body**. This is the very essence of **biodynamic embryology**: the anlage of structures (ears, ocular system, optic placode, etc.) is established according to this pattern.

We will add elements to this understanding, but after grasping this guideline.

A. LINKING EMERGING STRUCTURES TO MOVEMENT

Understanding this movement allows us to see how the **ventricles** are established, how the **dura mater** follows, how the **cranium** forms. If something blocks (a level I do not yet know), I can rely on my knowledge of timing and synchronous phenomena.

For example, I can look for an **artery** or a specified electrical level. The patient will be able to feel what is happening.

B. THE IMPORTANCE OF CARDIAC DYNAMICS

It is essential to have a good understanding of the heart's **dynamics**. Sometimes, expansion is greater on one side (for example, on the left).

C. AVOIDING EXCESSIVE INTELLECTUALISATION

It is important not to over-intellectualise the sensation. Too much mental activity can "put too much electricity into the limbic system" and hinder subtle perception.

V. PRACTICAL CASE: ANALYSIS OF A SENSATION

During an assessment:

- Expansion is gentle, not complete. Flexion is good but lacks dynamisation.

- The rhythm is variable. The movement shifts more to a track on the left, with a slowing down.
- It is sometimes difficult to reach the **frontal lobe** in depth.

A. PROGRESSION OF MOVEMENT

Then, we move to the pontine system, with an ascent of the patient's tube. I become a point of support, and it pushes. Lateral expulsion, particularly on the right, starts stronger.

The patient "recovers the package below". After a slightly slow initial start, the movement catches up well.

B. THE PATIENT'S TESTIMONY

The patient describes a difficult start, as if they had to "get going", release a fixation on the left that prevented expansion. Initial flexion was difficult.

When a connection was established, the system "unblocked". This is comparable to learning to drive a **Formula 1** car: at first, one is clumsy, then with learning, it works.

C. KEYS TO ACCOMPANIMENT

- **Environment and space:** Allow breathing, shift the movement.
- **Progressiveness:** Enter progressively and connect to this "writing" of the body.
- **Centring of listening:** Centre one's listening at the brain level, and if it "doesn't work", observe elsewhere.

It is a question of **synchronicity**. One must "create a synchronism" to find the right word and work with this synchronicity.